Gemcitabine and cisplatin induction before chemoradiotherapy

This chemotherapy is given before chemoradiotherapy to improve cancer control. Induction is the first stage of the planned pathway, and does not replace the later treatment.

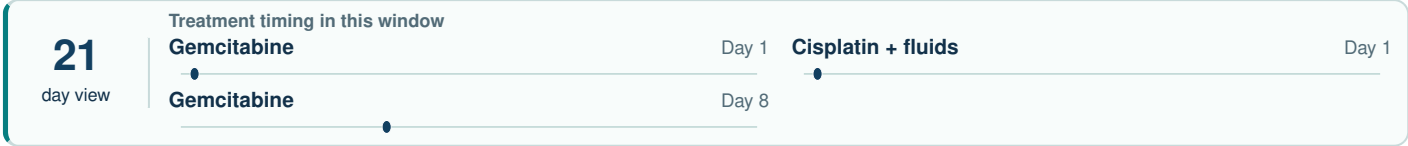
Head and neck: nasopharyngeal cancer · NCCP 00903 v1 · Document revised 11 November 2026

Your personal plan: this is the standard regimen schedule. Your team confirms your prescribed doses, cycles, treatment changes and appointments in your written plan.

The NCCP plan starts chemoradiotherapy 21–28 days after Day 1 of the last induction cycle, subject to review and recovery. This is measured from the first day, not from Day 8 or the end of that cycle. The later regimen has its own prescription.

My overall treatment

Day 1: both drugs; Day 8: gemcitabine. Every 21 days, up to 3 cycles.



Treatment

Gemcitabine 1,000 mg/m² on Days 1 and 8, with cisplatin 80 mg/m² on Day 1, every 21 days for a maximum of 3 cycles.

Review / next phase

The NCCP plan starts chemoradiotherapy 21–28 days after Day 1 of the last induction cycle, subject to review and recovery. This is measured from the first day, not from Day 8 or the end of that cycle. The later regimen has its own prescription.

A typical 21-day cycle

Day 1	Day 2–7	Day 8	Day 9–21
treatment	Between the first two visits	gemcitabine	Recovery and next review

Timing may change after review. Side effects can occur on any day; there are no guaranteed safe days.

My tests and checks

When	What is checked	Timing / reason
Before	**Baseline preparation** NCCP / individual plan	Full blood count, kidney and liver tests; review medicines, allergies, weight, oral intake and symptoms. Review nerve symptoms and hearing; audiology when indicated. Cisplatin eligibility includes adequate kidney function.
During	**Required tests** NCCP	Day 1: full blood count, kidney and liver tests. Day 8: full blood count. Audiology is arranged when clinically indicated; extra tests depend on symptoms and results.
After / response	**Recovery and next step** Individual plan	The team follows recovery, response and persistent side effects. Scans and tumour tests follow the individual plan; there is no universal scan or tumour-marker schedule in this regimen.

Between treatments

- Side effects can occur on any day. Keep prescribed support medicines and contact details to hand.
- Call promptly for fever or sudden illness, new breathing symptoms, persistent vomiting or inability to drink.

Bring your medicines list, appointment calendar and verified urgent contacts.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

Feasibility build. Clinical, pharmacy and patient/PPI review pending. Your team confirms your personal plan.

What to watch for

You do not need to know which medicine caused a symptom before calling.

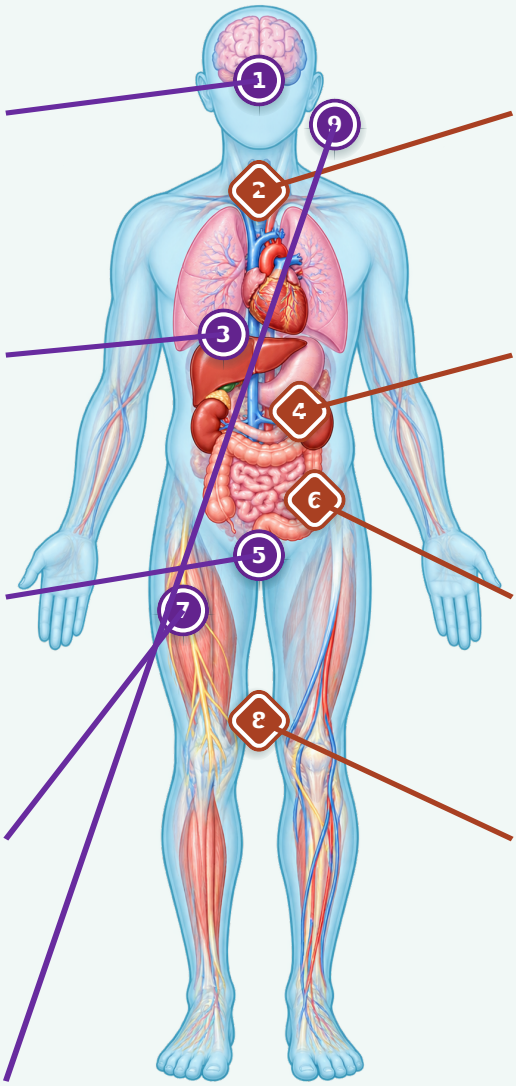
1 Fatigue and concentration
Gemcitabine + Cisplatin
Tiredness can affect activity and concentration. Balance activity and rest; report sudden confusion.

3 Breathing
Gemcitabine + Cisplatin
New or worsening breathlessness, cough or chest symptoms need urgent advice; lung inflammation can be serious.

5 Sickness and bowels
Gemcitabine + Cisplatin
Take prescribed anti-sickness medicines. Report persistent vomiting, diarrhoea, dehydration or inability to drink.

7 Skin and infusion reactions
Gemcitabine + Cisplatin
Report severe rash, blistering, facial swelling or breathing symptoms promptly. Tell staff about pain at the infusion site.

9 Hearing and nerves
Cisplatin
Report ringing, hearing loss, numbness or tingling before the next dose; some effects can persist.



2 Mouth and swallowing
Gemcitabine + Cisplatin
Report soreness, ulcers or difficulty swallowing. Ask early about mouth care, pain relief and nutrition.

4 Blood and infection
Gemcitabine + Cisplatin
Fever, shivering, sudden illness, unexplained bruising or bleeding needs urgent oncology advice.

6 Kidneys and fluid balance
Gemcitabine + Cisplatin
Report reduced urine, swelling or poor fluid intake. Blood tests help detect kidney injury.

8 Fertility and pregnancy
Gemcitabine + Cisplatin
Discuss fertility preservation, contraception, pregnancy and breastfeeding before treatment.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Fever, shivering, sudden illness, unusual bleeding, new breathlessness, persistent vomiting or inability to drink needs urgent oncology advice. Severe chest pain, severe breathing difficulty, collapse or a seizure is an emergency: call 112/999.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

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Keep your treatment details to hand

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Print this page **one-sided on A4 at Actual size / 100%**. Cut only the dashed outer border around both panels. **Do not cut the centre line.** Fold with FRONT and BACK facing out, then glue or tape the blank backs together. Finished size: **85.6 × 54 mm**. Keep the stamp side writable.

FRONT / CONTACTS

My Treatment Card

SACTCheck

Name: _____

Gemcitabine 1000 + cisplatin induction

Day 1: both drugs; Day 8: gemcitabine. Every 21 days, up to 3 cycles.

Phase: _____ Planned cycles: 3

Daytime (EXAMPLE) 087 12345678
Out of hours (EXAMPLE) 087 87654321

Unwell or fever? Call oncology now.
Severe breathing difficulty, chest pain or collapse: **112 / 999.**

EXAMPLE NUMBERS — replace before patient use.

CUT outer border

BACK / TREATMENT RECORD

My cycle record

Team stamps after each completed cycle.

C1	C2	C3
Date / initials	Date / initials	Date / initials

My treatment information

Plan changed? Ask the team to update this card.

FOLD centre — do not cut

Keep the record accurate

The team dates and initials each recorded treatment unit when complete. A cycle is complete after its final planned dose; a weekly-treatment stamp records that weekly dose. Delays, omitted doses or a changed plan need a note or a replacement card. A stamp does not approve the next treatment. Keep any medicine-specific alert card too.

Put your treatment page on your phone

Scan your QR and check the treatment name. Save that page to your home screen.

iPhone / Safari

Tap Share → Add to Home Screen. Select Open as Web App if offered, then Add.

Android / Chrome

Tap the three-dot menu → Add to Home screen or Install and create shortcut. Choose the shortcut option and confirm.

Menu wording varies. The QR opens information, not a personal record. Paper stamps do not update the website. Check that the saved icon opens the correct treatment.

Your local contacts and resources

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

Hospital / team: _____
Backup if the team cannot be reached: _____

Additional local services and resources will be added when confirmed. Your team must check and complete this page before patient use.

Feasibility build. Clinical, pharmacy and patient/PPI review pending. Your team confirms your personal plan.